

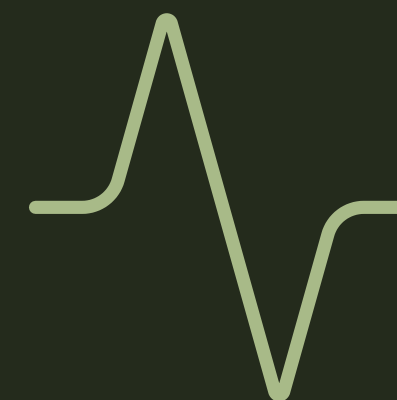
715.

identified.

invited.

counted.

the gap is 8.8 years,
and it has not closed.



Aboriginal and Torres Strait Islander men live 8.8 years less than other Australian men; women, 8.1 years less. Target 1 — close the gap in life expectancy within a generation, by 2031 — is not on track.

four of nineteen.



4 / 19

Closing the Gap targets on track (Annual Data Compilation, July 2026)



8.8 yrs

life expectancy gap for men; 8.1 years for women. Target 1 not on track



89.2%

of babies at a healthy birthweight, against a 91% target. Not on track



2024

Productivity Commission review: fundamental change is required

Productivity Commission, Closing the Gap Annual Data Compilation Report (July 2026); Review of the National Agreement on Closing the Gap (February 2024); AIHW Closing the Gap targets reporting.

this is not a plan to close the gap.

WHO DECIDES

Priority Reform One settles that: formal partnership and shared decision-making with Aboriginal and Torres Strait Islander people. That is not us.

WHO DELIVERS

Priority Reform Two settles that: the community-controlled sector. ACCHOs already outperform mainstream general practice for the people this concerns. That is not us either.

WHAT WE OFFER

Infrastructure and measurement — for a community-controlled organisation to use, adapt, or refuse.

We have no Aboriginal or Torres Strait Islander partner organisation today. Securing one, on their terms, is the first phase of the pilot proposed here — not a condition satisfied after funding is granted.

the front door is already paid for.

MBS item 715 — the annual Aboriginal and Torres Strait Islander health assessment — already exists, is already funded, and is the recognised entry point to chronic disease care. It is the one thing in this system that needs no new policy, no new money and no new clinical model.

27.9%

of Aboriginal and Torres Strait Islander people received a 715 health assessment in 2023–24

17%

in the lowest Primary Health Network areas

48%

of regular ACCHO clients aged 15 and over

Uptake figures 2023–24. National average and PHN range from Department of Health reporting; ACCHO regular-client rates from national key performance indicator reporting.

none of these are clinical problems.

IDENTIFICATION	Indigenous status is not recorded, so the list of eligible patients does not exist to begin with.
ITEM KNOWLEDGE	Practice staff do not know which MBS item applies, or who in the practice is entitled to claim it.
OWNERSHIP	Nobody owns the recall, so the health assessment is nobody's job in particular.
BILLING AVOIDANCE	Practices avoid claiming an item they are not confident they have satisfied the requirements for.

Barriers as identified in the published literature on 715 uptake. Each is an operations problem — which means it is measurable, and fixable without touching the clinical encounter.

a register, a recall, and a receipt.

Built over two years as domain-neutral primary-care infrastructure, currently running end to end against a synthetic practice engine.



01

Identify

A register of eligible patients built from the service's own records, inside rules it sets. Nothing leaves the service to build it.



02

Invite

A plain, availability-only message. The patient books with their own clinician, at their own service.



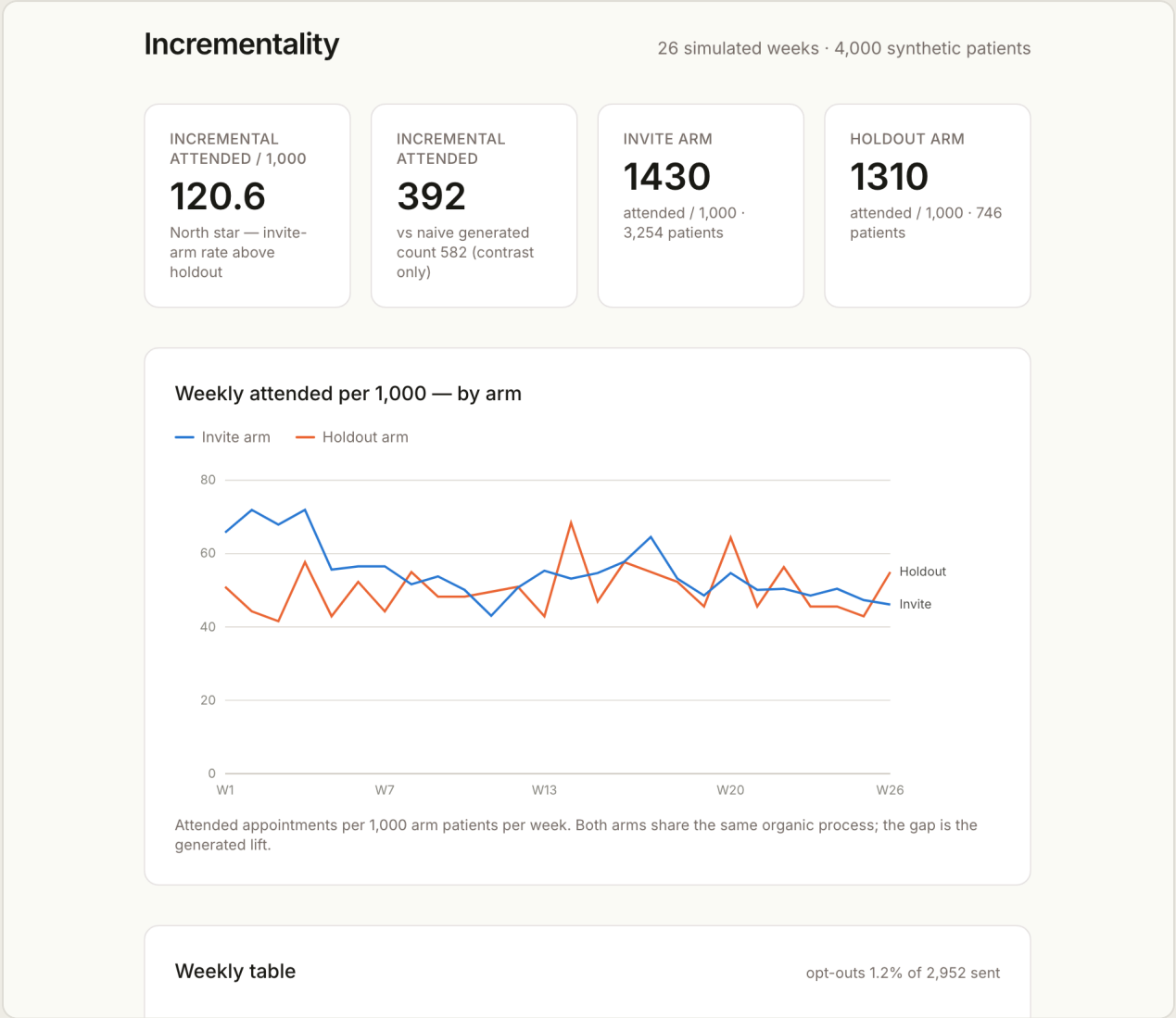
03

Count

Attended assessments only, against a randomised holdout. No-shows never count, and neither do bookings we merely touched.

it already reports
the counterfactual.

A randomised holdout arm runs continuously alongside the invited arm. The reported figure is the difference between them — not the number of people the program touched.



a program that can report zero.

WHAT PROGRAMS USUALLY REPORT

Activity. Messages sent, checks performed, dollars spent, people reached. Every one of those numbers counts something the program did — and not one of them answers whether it changed anything.

WHAT THIS REPORTS

Incremental attended assessments per 1,000 eligible patients, against a randomised control arm running continuously. If the program did nothing, the number is zero, and the report says zero.

The 2024 Review found that governments cannot show what is working. This is a literal answer to that: a number that is allowed to come back negative.

inside the Agreement, not beside it.

PRIORITY REFORM ONE

Shared decision-making

The partner organisation sets eligibility, message content, and whether the pilot proceeds at all. Written into the agreement rather than left to goodwill.

PRIORITY REFORM TWO

Community-controlled sector

Deployed as the partner's own infrastructure. We never hold the patient relationship, the clinical record, or the Medicare claim.

PRIORITY REFORM THREE

Transforming government

The same measurement is offered to mainstream practices — where Aboriginal and Torres Strait Islander patients most often are not identified at all.

PRIORITY REFORM FOUR

Shared access to data

The partner organisation is the data custodian. Nothing is disclosed, published or shared without its decision.

one region, twelve months, one auditable number.

PHASE 0 · MONTHS 1–3	Partnership first. No build, no data and no deployment until an Aboriginal Community Controlled Health Organisation has agreed scope, governance and data terms in writing.
PHASE 1 · MONTHS 4–9	Deployment inside partner services. Register, invitation and booking under the partner’s rules, with the randomised holdout running from the first day.
PHASE 2 · MONTHS 10–12	Independent read-out: incremental attended 715 assessments per 1,000 eligible patients, and a plain account of what did not work.
PUBLICATION	The result is published whether positive or null, with the partner as co-author, and the underlying data remains theirs.

Deliberately unpriced. Costing a program that has not yet been co-designed would contradict the partnership sequencing above; the budget is developed with the partner in Phase 0 and put to the Department before any build begins.

three of us. the fourth seat is not ours to fill.

			
Vikram Ganeshalingam	Dr Anubhav Saxena	Stefan Thottunkal	Vacant, deliberately
CO-FOUNDER	CO-FOUNDER · MBBS, FRACGP	CO-FOUNDER	ABORIGINAL AND TORRES STRAIT ISLANDER GOVERNANCE
Final-year MD candidate, Bond University	Practising GP · University of Sydney	NOURISH, Stanford Medicine · Health Systems Innovation Lab, Harvard T.H. Chan	A condition of this work proceeding — not an advisory seat added after funding.

four things, in this order.

-  An introduction to NACCHO or a state affiliate, so that partnership begins with the sector rather than with us.
-  Scoped pilot funding, released in two tranches — partnership first, build second.
-  Access to regional MBS 715 claim data, on Priority Reform Four terms set by the partner.
-  A named contact in the Department to hold us to the measurement standard we are proposing.

If the holdout shows no effect, that is the finding, and we will publish it.

thank you

Vikram Ganeshalingam

Dr Anubhav Saxena

Stefan Thottunkal

add contact email before sending

We acknowledge Aboriginal and Torres Strait Islander peoples as the Traditional Owners and Custodians of this country, and pay respect to Elders past and present. Any work described here proceeds only with, and under the authority of, the communities it concerns.